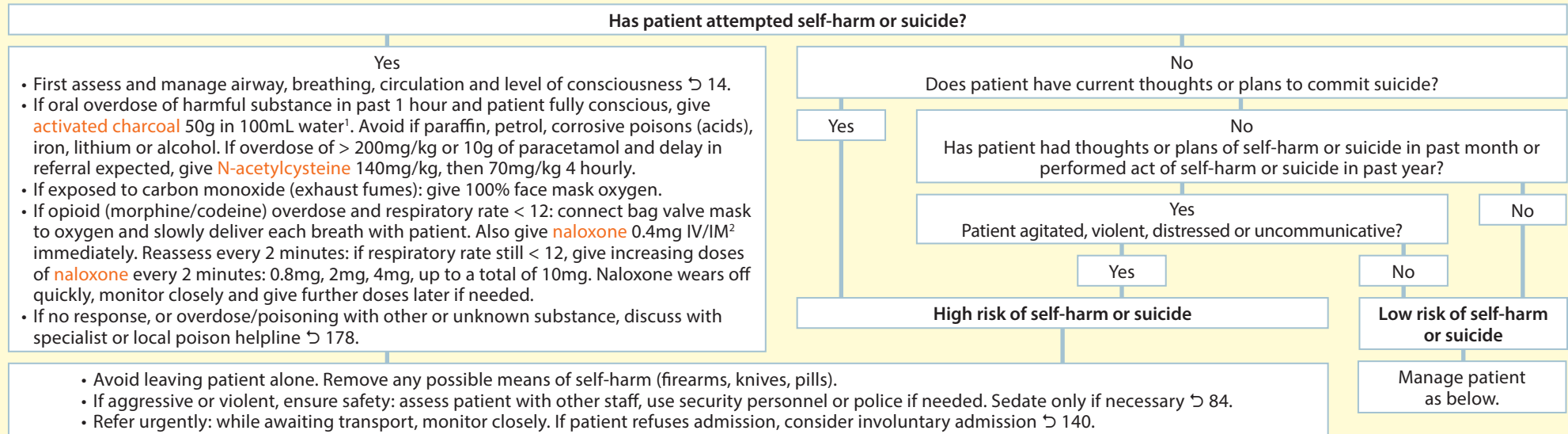


SELF-HARM OR SUICIDE

Give urgent attention to the patient who has attempted or considered self-harm or suicide:



Assess the patient whose risk of self-harm or suicide is low

Assess	When to assess	Note
Depression	Every visit	If known depression, give routine care ↗ 144, otherwise ask: in the past month, has patient: 1) felt down, depressed, hopeless or 2) felt little interest or pleasure in doing things? If yes to either ↗ 143.
Alcohol/drug use	Every visit	In the past year, has patient: 1) drunk ≥ 4 drinks ¹ /session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any ↗ 142.
Other mental illness	Every visit	• If hallucinations, delusions, disorganised speech, disorganised or catatonic behaviour, discuss with specialist same day. If memory problem, screen for dementia ↗ 148.
Stressors	Every visit	• If not known with a mental illness, assess for stress and anxiety ↗ 86. • Help identify psychosocial stressors. Ask about trauma, sexual abuse/violence ↗ 88, family or relationship problems, financial difficulty, bereavement, chronic ill-health.
Chronic condition	Every visit	• If chronic pain, assess and manage pain ↗ 60 and underlying condition. Link patient with helpline or support group ↗ 178. • If patient has a life-limiting illness, also consider giving palliative care ↗ 170.

Advise the patient whose risk of self-harm or suicide is low

- Discuss with patient reasons to stay alive. Encourage carers to closely monitor patient as long as risk persists and to bring patient back if any concerns.
- Advise patient and carers to restrict access to means of self-harm (remove firearms from house, keep medications and toxic substances locked away) as long as risk persists.
- Suggest patient seeks support from close relatives/friends and offer referral to counsellor or local mental health centre or helpline ↗ 178.
- Discharge into care of family, if possible. Review patient at least weekly for 2 months: involve a counsellor, psychiatric nurse/psychologist or social worker if possible.
- If self-harm or suicide risk is still low follow up monthly. If thoughts or attempts of self-harm or suicide recur, reassess suicide risk above.

¹If able, give this charcoal mixture via nasogastric tube if the airway is protected and patient co-operative. Charcoal may be useful if these poisons are taken in overdose: carbamazepine, barbiturates, phenytoin, dapsone, quinine, theophylline, salicylates, mushroom poisoning, slow release preparations, digoxin, beta-blockers, NSAIDs. ²Give naloxone IM only if IV not possible.